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EXPECTED SIZE OF INDIA'S HOME HEALTH-CARE MARKET BY 2027, AS PER A 2021 NITI AAYOG REPORT

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PLEASANT REVOLUTION IS TRANSFORMING HEALTHCARE IN INDIA. For a long time, home care was limited to end-of-life or palliative care, geriatric nursing and physiotherapy. All diagnostic, emergency and critical care functions happened in hospitals. Now, in a massive shift, the comfortable confines of a home are taking centre stage. They call it home healthcare or, simply, homecare. Today, complex clinical profiles can be managed at home, something deemed impossible a decade back. This includes looking after those in coma, stroke survivors and dementia patients. Other than the usual barrage of blood/urine tests, a wide range of diagnostic procedures like x-rays, ultrasounds, electrocardiograms (ECGs) and some biopsies can be done at home by trained professionals. Not just this, use of ventilators, dialysis units and setting up of e-ICUs (electronic intensive care units) have expanded the range of home healthcare. Doctors say it speeds up complex recoveries, including from organ transplants, cancer care and major surgeries. Furthermore, it saves money, reduces congestion in hospitals and is considered safer. Sensing a shift, major players like Apollo and Max hospitals have introduced homecare services under Apollo Home and MAX@Home, respectively, while other players like Portea Medical and Healthcare at Home (HCAH) contribute to a burgeoning market worth thousands of crores. Taking note, the government is not far behind.

The Covid-19 pandemic marked an evolutionary inflection point for home healthcare. According to government data, over 90 per cent of Covid cases were treated at home. "Increasingly, especially post Covid, patients prefer to recover in their homes. At Apollo, we saw this as both a need and an opportunity. Hence, we thought of creating a dedicated vertical for home healthcare," says Vishal Lathwani, CEO of Apollo Home, which has launched a service in Bengaluru that guarantees a home visit by a doctor within 90 minutes—the medical equivalent of quick commerce.

"Whether it's an elderly parent on long-term antibiotics, a cancer patient needing palliative support, or one regaining strength after a stroke, recovery at home leads to better clinical outcomes, higher emotional comfort, and lower risk of hospital-acquired infections," says Vikram Verma, senior VP & business head, MAX@Home.



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It has eased tensions for people like Swati Deshmukh, 63, who delayed her total knee replacement surgery for two years despite medical recommendation. She put it off because she was "terrified at being hospitalised for a week or so afterwards". Today, the former school teacher from Mumbai feels emboldened. "My sister-in-law opted for a hospital knee replacement surgery with homecare recovery. She was in hospital for just two days," says Deshmukh. "I now plan to get mine done."

The setting up of eICUs by major healthcare providers—with systems using video conferencing, high-definition audio and software tools for patient monitoring, data access and real-time support by ICU specialists—are a boon for many. MAX@Home installed more than 1,400 step-down ICUs—a stripped down ICU designed for homes—in 2024. A sample study of 506 patients by HCAH who used eICU-